

Greenleaf Medical Associates

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Type 2 Diabetes Management Care Plan

Patient Name:	Robert L. Roman
Date of Birth:	September 18, 1960
MRN:	GMA-0047821
Primary Physician:	Samantha L. Wu, MD
Plan Date:	January 28, 2026
Next Review:	October or November 2026 (to be confirmed by office)
Emergency Contact:	Gerald M. Roman (daughter), (410) 555-2819
Secondary Contact:	Grace Roman (wife, RN), (410) 555-3274

Diagnosis Summary

Type 2 Diabetes Mellitus (ICD-10: E11.9) - diagnosed 2018. Peripheral neuropathy, mild, bilateral lower extremities (ICD-10: E11.40). Chronic fatigue, etiology multifactorial. Blood pressure: controlled, no hypertension diagnosis at this time. Thyroid function: normal (labs reviewed January 2026).

Current Medications

Metformin 500mg:	One tablet twice daily with morning and evening meals. Do not skip doses. If dose is missed at mealtime, take with the next meal; do not double up.
Aspirin 81mg:	One tablet daily (low-dose cardiovascular prophylaxis)
Vitamin D3 2000 IU:	One capsule daily with a meal

Target Values

HbA1c Target:	Below 7.5% (current: 7.4% as of January 2026)
Fasting Glucose Target:	80 to 130 mg/dL (current: 127 mg/dL as of January 2026)
Blood Pressure Target:	Below 130/80 mmHg (current: 128/82)
Weight Goal:	Maintain or reduce toward 195 lbs (current: 199 lbs)

Foot Care Protocol (Peripheral Neuropathy)

Inspect both feet daily, preferably in good lighting. Look for redness, cuts, blisters, sores, or

swelling. Apply Eucerin Original Healing Cream to both heels and soles twice daily (morning and before bed). Wear cushioned, properly fitted shoes at all times; no walking barefoot on hard surfaces. Custom orthotics are in use (prescribed by Baltimore Podiatry Group, July 2024). Report any new skin breakdown, discoloration, or change in sensation to the office immediately. Annual podiatry assessment is recommended; next podiatry visit due January 2027.

Dietary Guidelines

Focus on consistent carbohydrate intake across meals rather than elimination. Aim for 45 to 60 grams of carbohydrate per main meal. Increase fiber from vegetables, legumes, and whole grains. Reduce refined sugars and sugar-sweetened beverages. Portion control at main meals. Evening snacks should be protein-based if hunger is present (e.g., a small handful of nuts). Sodium: aim below 2,300 mg per day. Patient should discuss specific changes with Dr. Wu before beginning any structured diet program.

Physical Activity

A short walk of 15 to 20 minutes after dinner is beneficial for glucose management and is recommended when energy permits. No high-impact exercise without physician review given neuropathy status. Gentle range-of-motion exercises or chair-based stretching on days when walking is not possible.

Monitoring

Blood glucose self-monitoring is not currently prescribed. HbA1c and fasting glucose are monitored at each quarterly appointment. Patient's daughter Gerald Roman maintains an appointment and medication log and accompanies the patient to most quarterly visits. Results will be communicated to both Gerald and Grace Roman per the patient's standing authorization on file.

Authorized by: Samantha L. Wu, MD | Greenleaf Medical Associates | January 28, 2026

This care plan is for informational guidance. It does not replace individualized medical advice from the treating physician.